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March 3, 2026

Yun Jiang, PhD, MS, RN, FAMIA and Jinjiao Wang, PhD, RN, MPhil
Co-Editors-in-Chief
JMIR Aging

Dear Editor:

I wish to submit an original paper for publication in JMIR Aging, titled "Development and Temporal External Validat Interpretable Machine Learning Model for Predicting 6-Month Mortality in Long-Term Care Facilities: A Retrospective paper was coauthored by Yang YC, WYL, Yu YC, TYC, CYW (Dept of Nursing), CYW (Jubo Health Technologies).

The bulk of machine learning models for mortality prediction remain dependent on acute-care hospitalization data, data tend to be inaccessible or significantly delayed during long-term care facility (LTCF) admission. There also is a driven prognosis in Asian LTCF contexts. This study aimed to develop and temporally externally validate an interpret model for predicting 6-month all-cause mortality among older adults newly admitted to LTCFs in Taiwan using rout data. We believe that our study makes a significant contribution to the literature because it underscores how an ex learning model built upon a parsimonious set of features can achieve accurate and generalizable predictions of 6-m term care facility residents. The data from an Asian population also helps add more region-specific data to the liter our knowledge of Asia-specific prediction processes and specificities for mortality in the context of LTCF admission.

Further, we believe that this paper will be of interest to the readership of JMIR Aging because it delivers much need and scientific evidence on the usability of machine learning for mortality predictions within the context of LTCFs. C aging population phenomenon and the fact that the coming years and decades tend only to see it worsen, having a generalizable enhancer of mortality prediction that uses data that is actually collected during initial geriatric assess help various stakeholders, from researchers to clinicians and other healthcare practitioners, with devising actionab to help prevent deaths upon LTCF admission. A potential outcome is improved end-of-life care and prevention of d high risk of complications upon LTCF admission. Those at a low risk may also benefit from targeted interventions t personal needs more adequately, leading to potentially less intrusive healthcare procedures and enhanced care sat

This manuscript has not been published or presented elsewhere in part or in entirety and is not under consideratio study participants provided informed consent, and the study design was approved by the appropriate ethics review and understood your journal's policies, and we believe that neither the manuscript nor the study violates any of th conflicts of interest to declare.

We acknowledge that an APF of \$2500 is payable upon acceptance.

Thank you for your consideration. I look forward to hearing from you.

Sincerely,

Yu-Tai Lo

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